

ARYABHATA EDUCATIONAL & HEALTH TRUST

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DISTRICT PROGRAMME CONTINUITY SCAN

Convergence & Decision-Support Framework

ASSESS • CONVERGE • PRIORITISE • DECIDE
ADMINISTRATIVE APPROVAL SUBMISSION • PHASE 1 PILOT

Purpose: A rapid, non-punitive, zero-PII pilot to help District Administration understand beneficiary pathways, inter-department convergence gaps, and priority actions across Education, Health/RBSK and Women & Child Development/Anganwadi delivery systems.

Document	Details
Document Type	Detailed Project Report / Pilot Proposal
Prepared For	District Administration
Pilot Scope	Up to 10 delivery points across the Education-Health/RBSK-WCD triad
Pilot Duration	5-7 working days, subject to access and district coordination
Financial Ask to District	Nil – proposed as an in-kind, no-cost pilot by Aryabhata
Version	Submission-ready corrected edition September 2026

Prepared by Aryabhata Educational & Health Trust

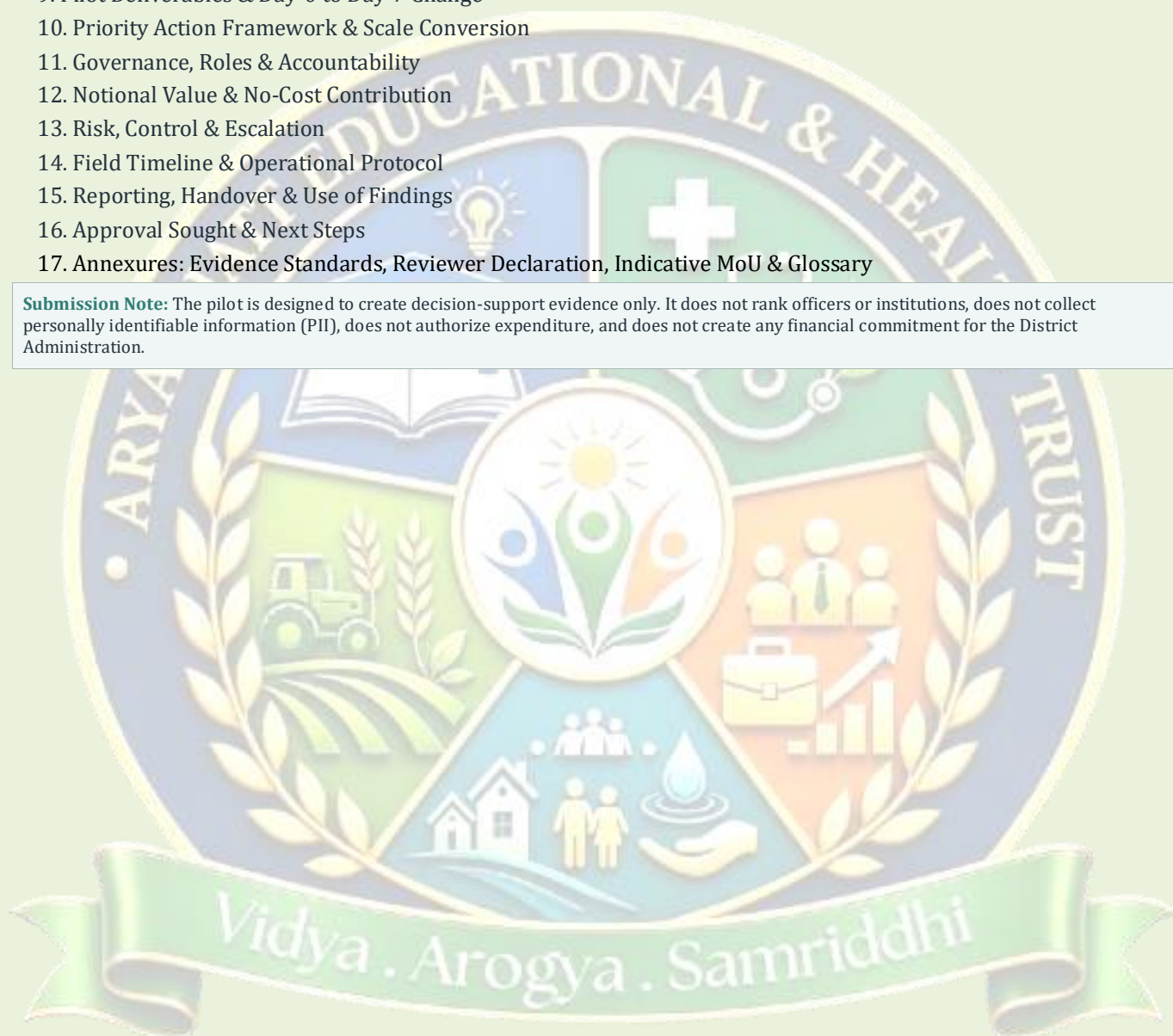
Pilot commencement date and final delivery-point list to be confirmed by the District Administration.

Vidya . Arogya . Samriddhi

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Submission Note: The pilot is designed to create decision-support evidence only. It does not rank officers or institutions, does not collect personally identifiable information (PII), does not authorize expenditure, and does not create any financial commitment for the District Administration.



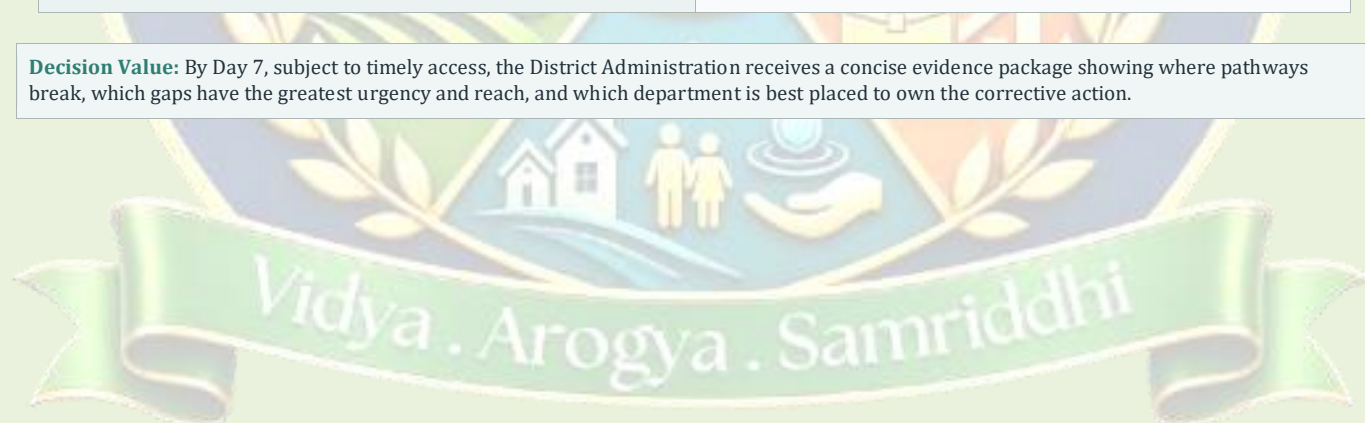
1. Executive Summary

District programmes often operate through different departments, reporting systems and delivery points. This can make it difficult for district leadership to see whether a beneficiary experiences a complete pathway across services. The District Programme Continuity Scan is a short-duration, no-cost pilot designed to provide a consolidated, evidence-based view of service continuity and convergence gaps without creating a parallel inspection system.

Phase 1 is deliberately limited to the Education–Health/RBSK–Women & Child Development (Anganwadi) triad. Up to 10 representative delivery points will be selected with the District Administration. The pilot will examine beneficiary pathways, institutional readiness, inter-department linkage, early outcome-readiness signals and sustainability conditions.

Pilot Feature	Submission-ready Position
Administrative intent	Decision support, not inspection or ranking
Coverage	Up to 10 representative delivery points across the triad
Data model	Zero-PII for Aryabhata; district/institutions retain any identity mapping
Core metric	Abhisaran Continuity Score (ACS), 0–100, with transparent evidence rules
Outputs	Impact Passports, convergence heat-map/dashboard, Priority Action Framework, exit briefings
Independent assurance	Methodology and anonymized outputs reviewed by an independent reviewer with conflict-of-interest declaration
Cost to District	Nil for the pilot; Aryabhata bears the stated notional in-kind cost
Next-step boundary	Any scale-up, funding, procurement or implementation requires separate due process and formal approval

Decision Value: By Day 7, subject to timely access, the District Administration receives a concise evidence package showing where pathways break, which gaps have the greatest urgency and reach, and which department is best placed to own the corrective action.



2. Introduction & Context

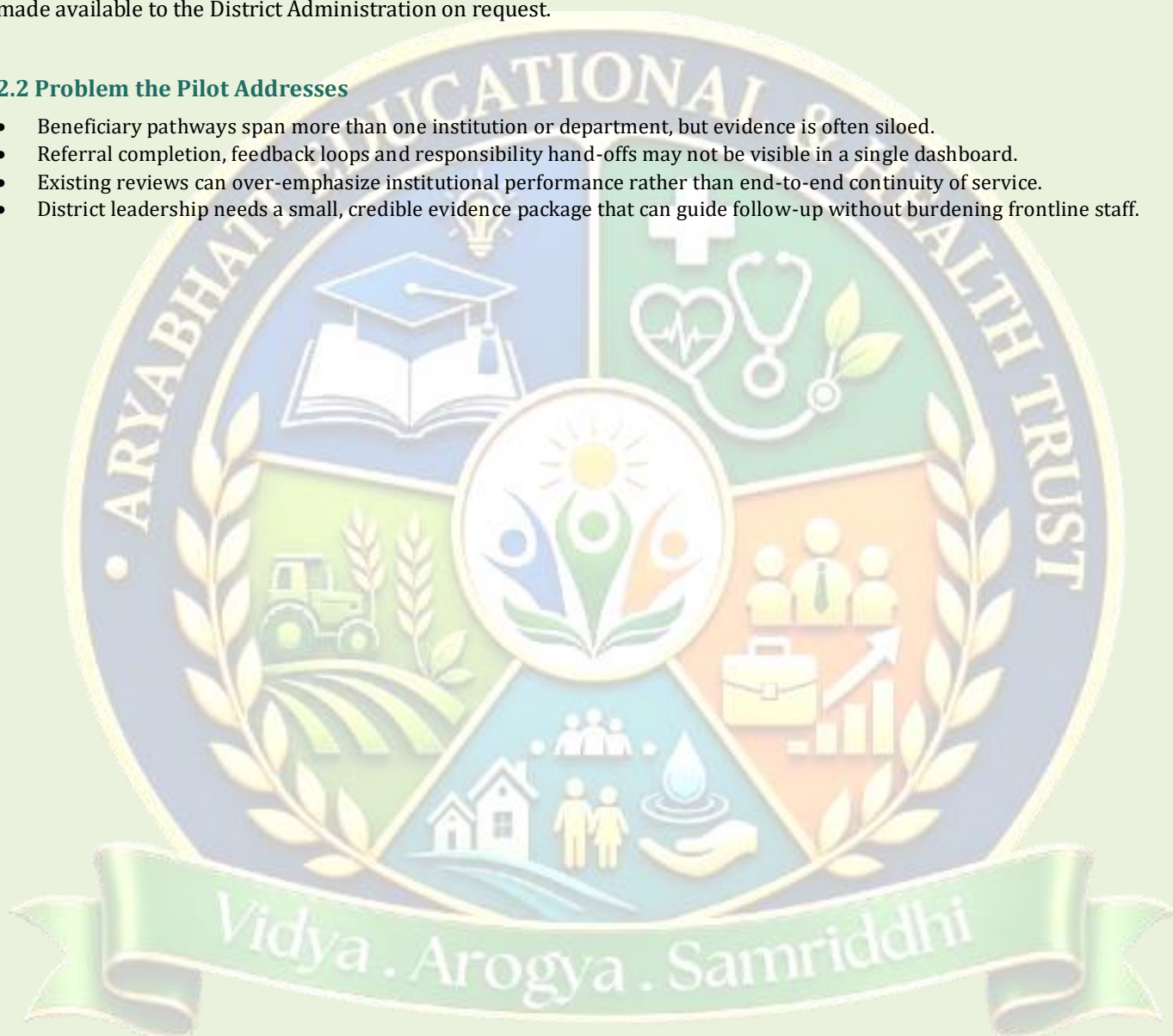
This proposal outlines a pragmatic, rapid approach to programme continuity assessment across selected district delivery systems. Its core problem statement is simple: district leadership may have programme-specific reports, yet still lack a consolidated view of how multiple departments converge around the same beneficiary journey.

2.1 Organisational Context

Aryabhata Educational & Health Trust states that it was established in July 2010 and is registered under 12A, 80G and CSR-1. The Trust reports field experience across education, health and livelihoods in Jharkhand, including work involving government departments and multi-sector programme delivery. Self-attested copies of the Trust Deed, 12A registration, 80G registration, CSR-1, PAN, and a summary of the last three years' audited financial statements will be submitted as separate enclosures and not stapled into the DPR. Current governance and authorized-signatory credentials will also be made available to the District Administration on request.

2.2 Problem the Pilot Addresses

- Beneficiary pathways span more than one institution or department, but evidence is often siloed.
- Referral completion, feedback loops and responsibility hand-offs may not be visible in a single dashboard.
- Existing reviews can over-emphasize institutional performance rather than end-to-end continuity of service.
- District leadership needs a small, credible evidence package that can guide follow-up without burdening frontline staff.

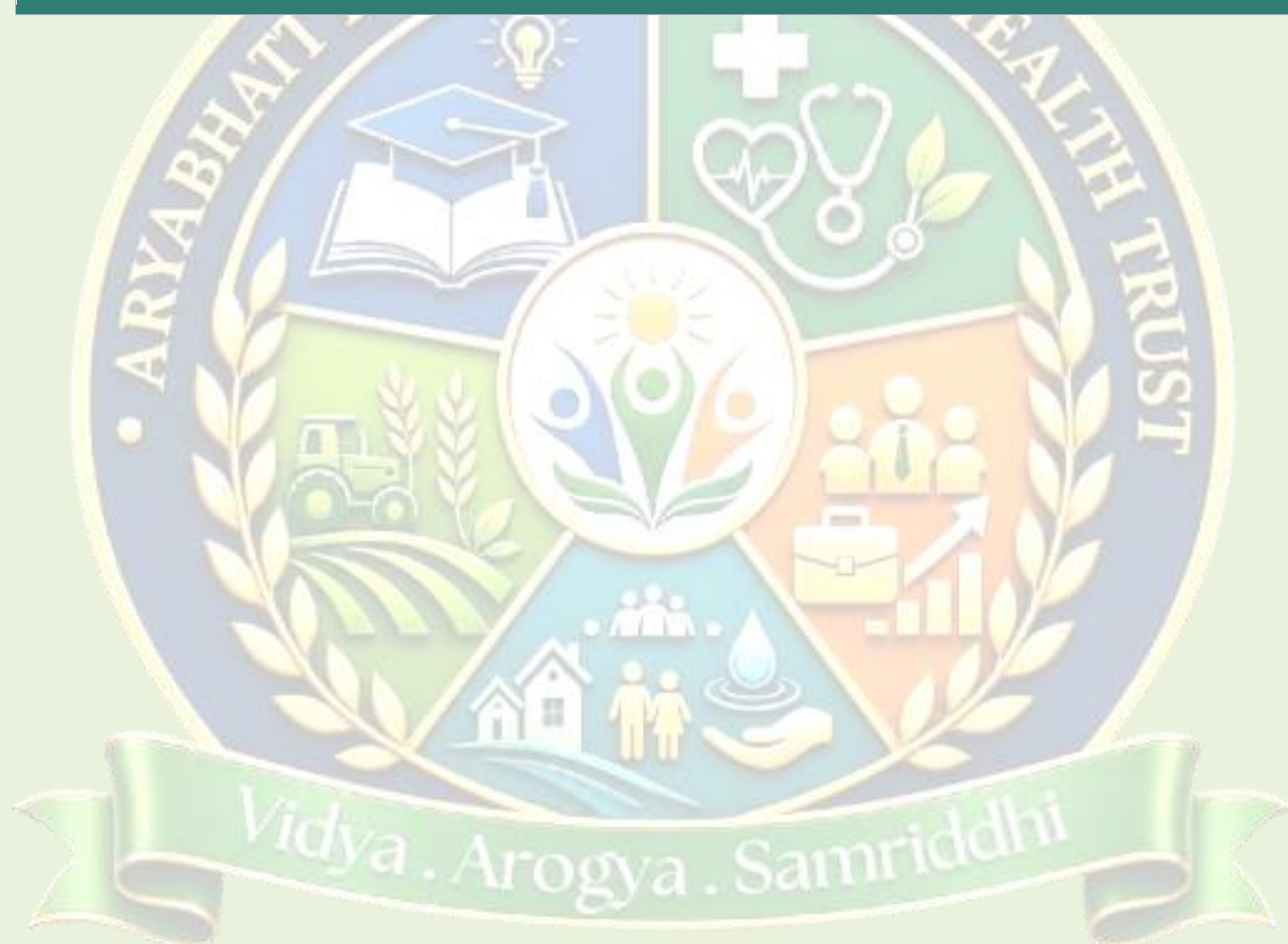


3. Pilot Positioning: What This Is / Is Not

This Pilot IS	This Pilot IS NOT
A collaborative process-mapping exercise	A school ranking or institutional league table
A beneficiary-pathway continuity scan	An officer appraisal mechanism
A time-bound evidence and decision-support exercise	A punitive inspection or vigilance exercise
A cross-sector convergence diagnostic	A substitute for statutory audit, inspection or departmental review
A zero-PII analytical exercise for Aryabhata	A beneficiary database or identity-tracking system

The focus shifts from “How is this institution performing?” to “What is the complete pathway a beneficiary experiences, where does continuity weaken, and what system-level action can address the gap?”

THREE NON-NEGOTIABLE ADMINISTRATIVE ASSURANCES



4. Jurisdiction, Scope & Sample Composition

The Abhisaran framework is designed for cross-department use; however, this DPR seeks approval only for a limited Phase 1 pilot. References to district-wide or multi-sector application describe the framework's future potential and do not expand the scope of this pilot.

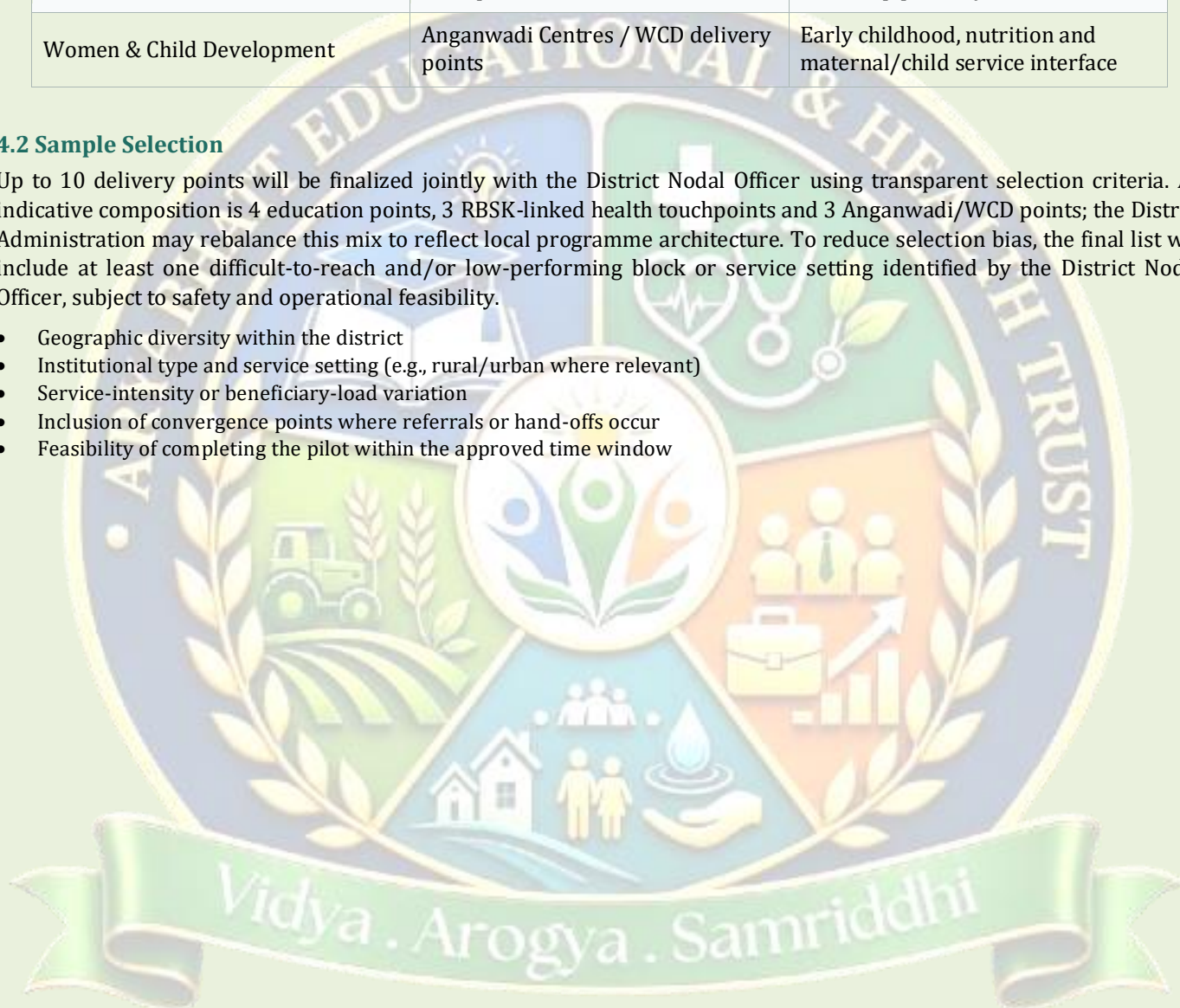
4.1 Phase 1 Scope

Sector / System	Indicative Delivery Points	Why Included
Education	Schools / education delivery points	Frequent child contact and referral interface
Health / RBSK	RBSK-linked screening/referral touchpoints	Child health screening, referral and follow-up pathway
Women & Child Development	Anganwadi Centres / WCD delivery points	Early childhood, nutrition and maternal/child service interface

4.2 Sample Selection

Up to 10 delivery points will be finalized jointly with the District Nodal Officer using transparent selection criteria. An indicative composition is 4 education points, 3 RBSK-linked health touchpoints and 3 Anganwadi/WCD points; the District Administration may rebalance this mix to reflect local programme architecture. To reduce selection bias, the final list will include at least one difficult-to-reach and/or low-performing block or service setting identified by the District Nodal Officer, subject to safety and operational feasibility.

- Geographic diversity within the district
- Institutional type and service setting (e.g., rural/urban where relevant)
- Service-intensity or beneficiary-load variation
- Inclusion of convergence points where referrals or hand-offs occur
- Feasibility of completing the pilot within the approved time window

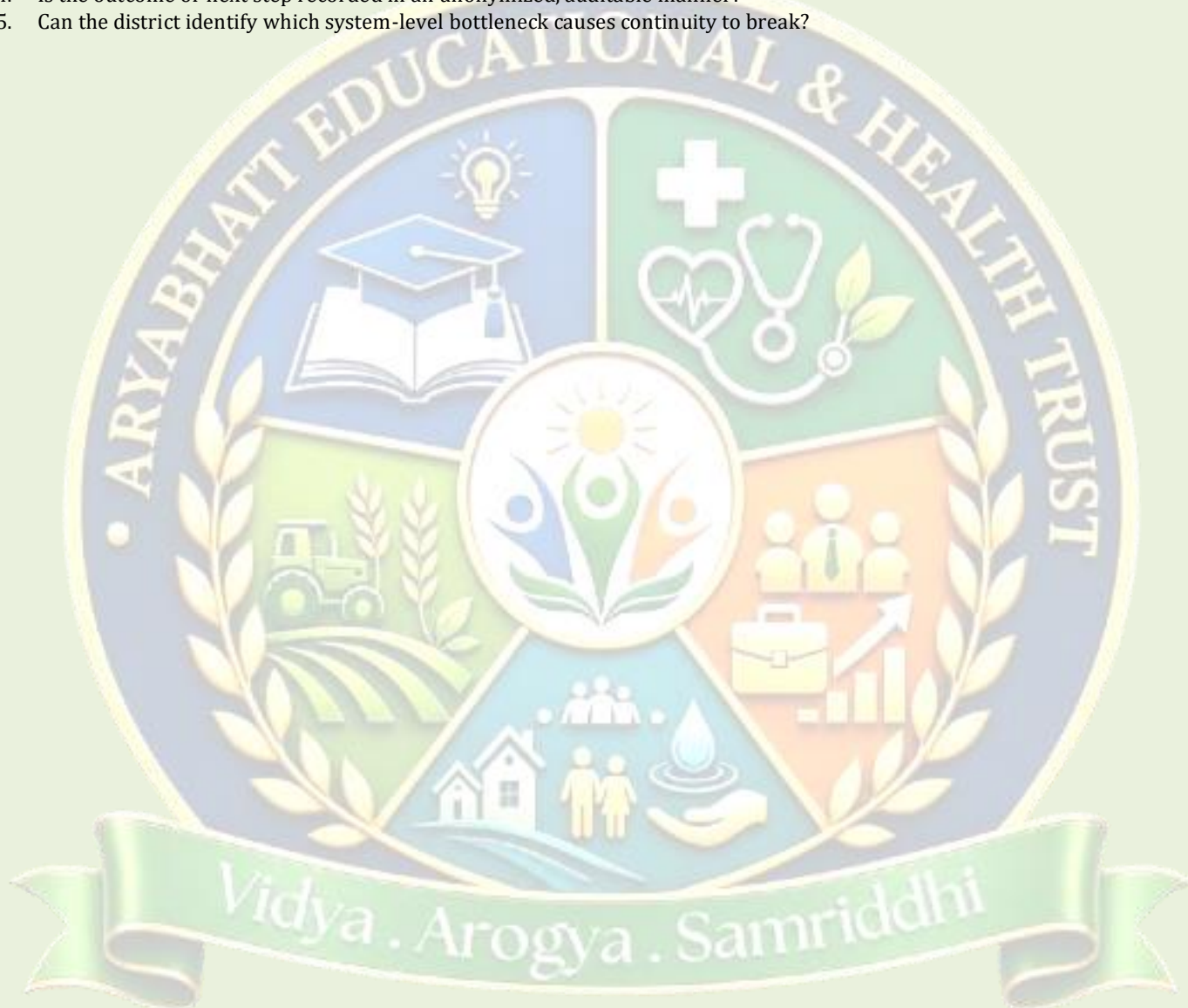


5. Abhisaran Convergence Model

Abhisaran (अभिसरण) means convergence. The framework maps how departments and delivery systems connect around beneficiary pathways. For Phase 1, the operational assessment is restricted to Education, Health/RBSK and WCD/Anganwadi. The broader framework can later be extended, subject to separate approval, to other district functions such as water and sanitation, rural development, tribal/social justice, agriculture/food security, and skill development/employment.

5.1 Convergence Questions

1. Is the required service or referral step documented?
2. Does the receiving institution acknowledge or complete the referral?
3. Is the hand-off completed within a reasonable programme-defined time?
4. Is the outcome or next step recorded in an anonymized, auditable manner?
5. Can the district identify which system-level bottleneck causes continuity to break?



6. Methodology: Five Layers of Assessment

Field teams use Collaborative Process Mapping to identify system bottlenecks rather than individual failures. Evidence is collected from permitted observation, institutional records made available by authorized officials, process documents, anonymized referral records, and structured discussions with designated staff.

Layer	What is Assessed	Examples of Evidence
1. Beneficiary Experience	Actual touchpoints and pathway completeness	Anonymized pathway steps, referral status, service sequence
2. Institutional Readiness	Staffing, equipment, protocols and operating conditions relevant to the pathway	Duty/availability records, protocol display, functional readiness checks
3. Departmental Alignment	Linkage within and across participating systems	Referral formats, nodal contacts, acknowledgement / feedback loop
4. Outcome-Readiness	Early signals that the pathway can produce intended change	Completed referrals, closure status, follow-up evidence
5. Sustainability	Capacity to maintain the improved pathway after the pilot	Ownership, routine review mechanism, feasible corrective action



7. Abhisaran Continuity Score (ACS)

The ACS is a 0–100 continuity score intended to summarize linkage strength across four equally weighted components. The score is a decision-support indicator, not an institutional performance grade.

Component	Weight	Evidence Test
Documented referral / hand-off	25	Evidence that the required referral or inter-service hand-off is formally recorded.
Follow-up completion	25	Evidence that the next responsible service point acknowledged or completed the required action.
Timeliness	25	Evidence that the action occurred within the relevant programme norm or locally approved benchmark.
Outcome / closure documentation	25	Evidence that closure, next step, or unresolved status is recorded in an anonymized form.

7.1 Scoring Rule

Each component is scored on a 0–5 evidence scale and converted to its weighted contribution. ACS = sum of weighted component scores across applicable components. If a component is legitimately not applicable, the denominator is re-based across the remaining applicable components and the dashboard must display the number of applicable components.

Evidence Score	Interpretation
0	No evidence / pathway absent
1	Ad hoc or anecdotal evidence only
2	Partial process exists; weak documentation
3	Process usually present; some gaps
4	Strong and repeatable evidence with minor gaps
5	Complete, timely and documented continuity

ACS Band	Interpretation
70–100	GREEN – Strong continuity
40–69	AMBER – Developing continuity
0–39	RED – Needs attention

Safeguard: ACS bands must not be published as school, officer or institution rankings. They are used to locate pathway-level bottlenecks and guide corrective action.

8. Data Governance, Privacy & Independence Safeguards

8.1 Zero-PII Commitment

Aryabhata will not collect, receive, store, process or retain directly identifying information of children, beneficiaries or individual staff for this pilot. Where continuity tracking is necessary, the District IT cell or an authorized participating institution may issue non-identifying continuity tokens. Aryabhata will not have access to the token-to-identity mapping, will not attempt re-identification, and will treat all tokenized records as restricted and sensitive project data under controls aligned with the Digital Personal Data Protection Act, 2023 and applicable government instructions. The district/institution alone retains any token-to-identity mapping; Aryabhata receives only the minimum tokenized or pre-aggregated fields authorized for the pilot.

PII Breach Protocol: If any personally identifiable information is accidentally received, viewed, copied or transmitted to Aryabhata, the recipient will stop further processing, securely delete or return the material as technically appropriate, preserve only a non-identifying incident record, and intimate the District Nodal Officer within two hours of discovery. Any further action will follow the District Administration's direction and applicable law/policy.

8.2 Data Protection Controls

- Data minimisation: collect only fields necessary for the approved assessment questions.
- Aggregation: dashboard views use aggregate or tokenized records; no beneficiary identity fields.
- Access control: pilot working files limited to authorized team members on a need-to-know basis.
- Retention: tokenized/anonymized working files will be retained only for the approved reporting and validation period and, unless the District Administration directs otherwise in writing, securely deleted within 30 days after the Day-7 handover. Aryabhata will submit a deletion certificate to the District Nodal Officer after completion of deletion.
- No public disclosure: findings are shared with authorized district officials; any wider publication requires written approval and additional privacy review.
- Applicable law/policy: work will follow applicable data-protection law, child-safeguarding requirements and district IT/security instructions.

8.3 Independent Quality & Evidence Reviewer

An independent reviewer will conduct a desk-based review of the methodology and data-collection protocol before field deployment, and validate anonymized analytical outputs and the Priority Action Framework after fieldwork. The reviewer will not shadow frontline staff during routine delivery unless separately authorized by the District Administration. Any reviewer honorarium or other financial relationship, if applicable, will be disclosed in writing to the District Nodal Officer before confirmation of the reviewer.

- Relevant academic/public-policy/health/education/M&E expertise
- No direct reporting relationship to the field team for this assignment
- Signed conflict-of-interest and confidentiality declarations
- Right of the District Administration to accept, propose an alternative, or veto the reviewer

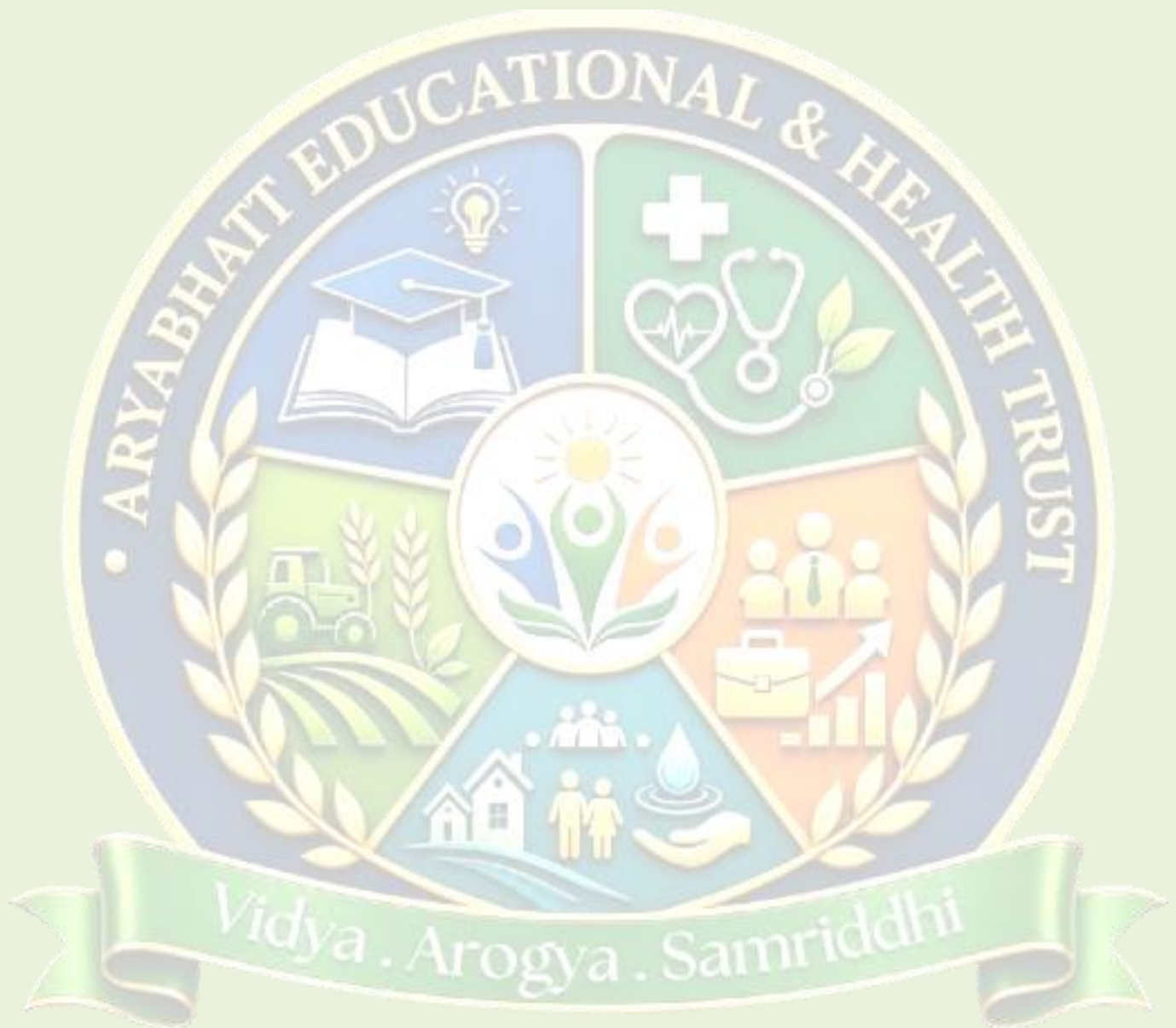
Independence eligibility: the reviewer will not be a current employee, board member, donor, or paid consultant of Aryabhata, and will not have held such a role during the preceding three years. Any potential conflict will be declared before appointment. Reviewer CV and signed Conflict-of-Interest (COI) declaration will be attached as Annexure D / separate reviewer enclosure for District review.

9. Pilot Deliverables & Day-0 to Day-7 Change

Deliverable	Quantity / Form	Purpose
Impact Passport	One concise summary per assessed delivery point (up to 10)	Records pathway observations, verified gaps, strengths and action ownership without ranking.
District Convergence Dashboard	One district-level analytical view	Shows aggregate heat-map, ACS bands, pathway breaks and cross-sector patterns.
Priority Action Framework	One prioritized action register	Ranks verified gaps by urgency, reach and feasibility; identifies responsible department/system.
Exit Briefings	At each participating delivery point plus district-level handover	Shares factual observations transparently and enables correction of factual errors before finalization.

9.1 Day 0 to Day 7 Change

Before Pilot	After Pilot
No consolidated cross-sector continuity view	District-level convergence dashboard
No standardized pathway summary	Up to 10 Impact Passports
Gaps not consistently prioritized	Priority Action Framework using common scoring rules
Limited evidence package for follow-up	Verified, anonymized evidence package with ownership and next-action fields



10. Priority Action Framework & Scale Conversion

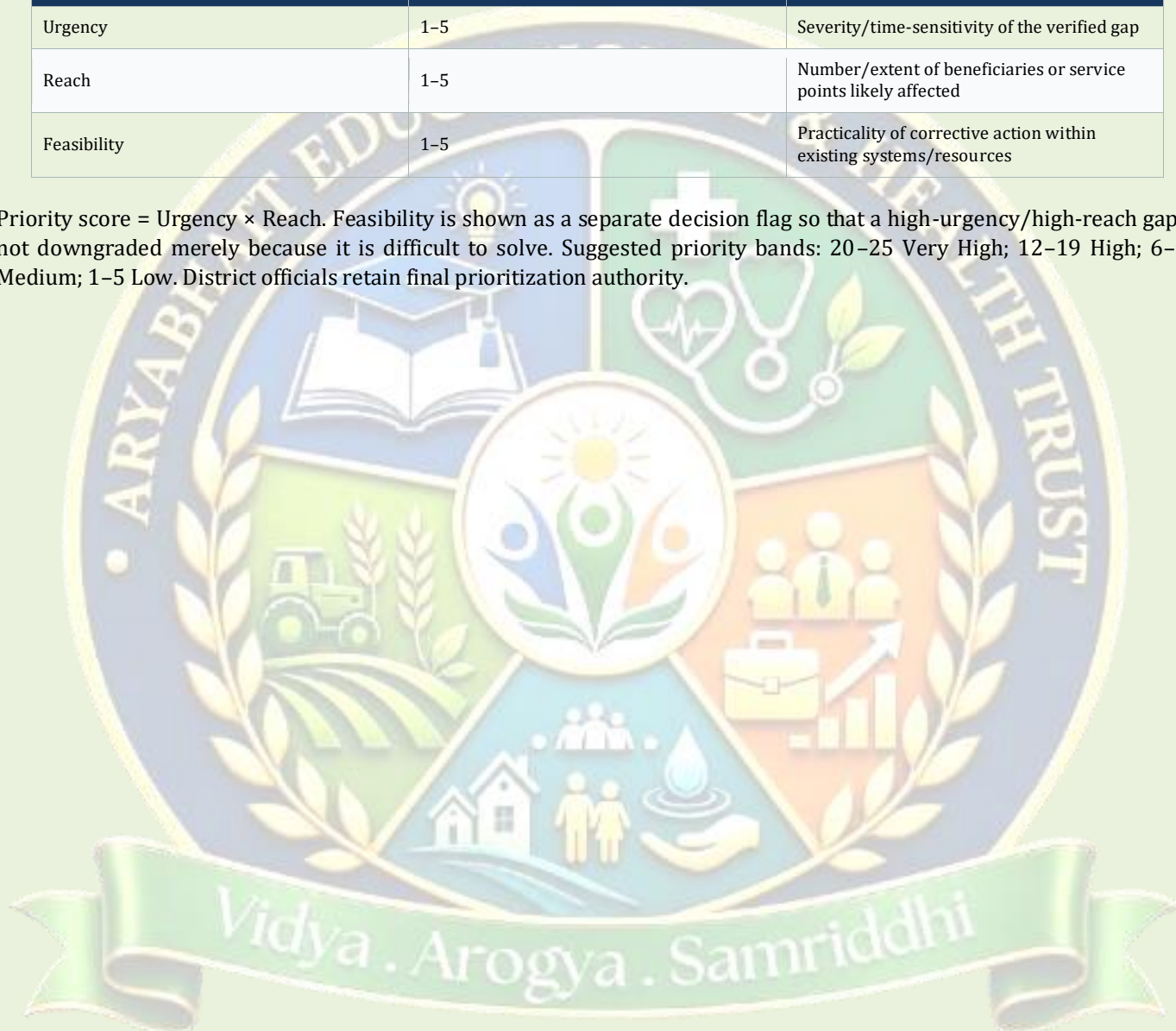
Verified gaps are translated into decision-ready action entries. For high-priority issues, the pilot may prepare a short concept-note style action brief containing problem description, beneficiary impact, indicative resource need, indicative timeline, responsible department/system and required next approval.

Important boundary: These action briefs are planning inputs only. They do not constitute automatic approval, sanction, procurement authorization, or guaranteed eligibility under DMF/PMKKKY/CSR or any other funding source. Any funding route remains subject to applicable scheme rules, competent-authority approval, technical estimates and due process.

10.1 Priority Scoring

Factor	Scale	Meaning
Urgency	1-5	Severity/time-sensitivity of the verified gap
Reach	1-5	Number/extent of beneficiaries or service points likely affected
Feasibility	1-5	Practicality of corrective action within existing systems/resources

Priority score = Urgency × Reach. Feasibility is shown as a separate decision flag so that a high-urgency/high-reach gap is not downgraded merely because it is difficult to solve. Suggested priority bands: 20–25 Very High; 12–19 High; 6–11 Medium; 1–5 Low. District officials retain final prioritization authority.



11. Governance, Roles & Accountability

Role	Core Responsibility	Decision Rights
District Magistrate / Deputy Commissioner	Administrative oversight and approval of pilot conditions	Approves pilot, nominates nodal officer, determines use of findings
District Nodal Officer	Day-to-day coordination, access facilitation, schedule alignment	Confirms sample, resolves field coordination issues, validates factual corrections
Aryabhata Field Team	Evidence collection, process mapping, anonymized analysis and draft outputs	No sanctioning or disciplinary authority
Independent Reviewer	Methodology/evidence assurance and analytical challenge	Provides review note; no implementation authority

11.1 Government Ownership Architecture

- District Administration owns administrative decisions and determines authorized use of findings.
- Source records remain with the competent government institution/department.
- Aryabhata provides methodology, field evidence synthesis and agreed deliverables only.
- Any future implementation support requires a separate written scope and approval.



12. Notional Value & No-Cost Contribution

The total notional in-kind value of the pilot is ₹5.20 lakh. Aryabhata proposes to bear this cost for the approved pilot. No reimbursement, grant, procurement commitment, recurring financial liability or automatic continuation is sought from the District Administration under this DPR. Submission, approval or completion of this pilot does not make Aryabhata eligible for preferential consideration for any future DMF, PMKKKY, CSR, departmental or other funding opportunity.

Cost Head	Notional Value (₹ lakh)	Basis / Note
Field assessment team	2.10	Team deployment, field coordination and assessment effort
Data analysis & dashboard build	1.20	Data cleaning, anonymized analysis and dashboard preparation
Impact Passport development	0.90	Delivery-point synthesis and action documentation
Independent review	0.60	Pre/post desk review, conflict check and review note
Reporting & presentation	0.40	Final report packaging and district handover
TOTAL	5.20	In-kind contribution by Aryabhata; nil financial ask to District

Financial Safeguard: Any request arising from the findings—whether through departmental budgets, DMF/PMKKKY, CSR, or another source—must be processed separately under the applicable rules. This pilot creates no right or expectation of subsequent funding to Aryabhata or any third party.



13. Risk, Control & Escalation

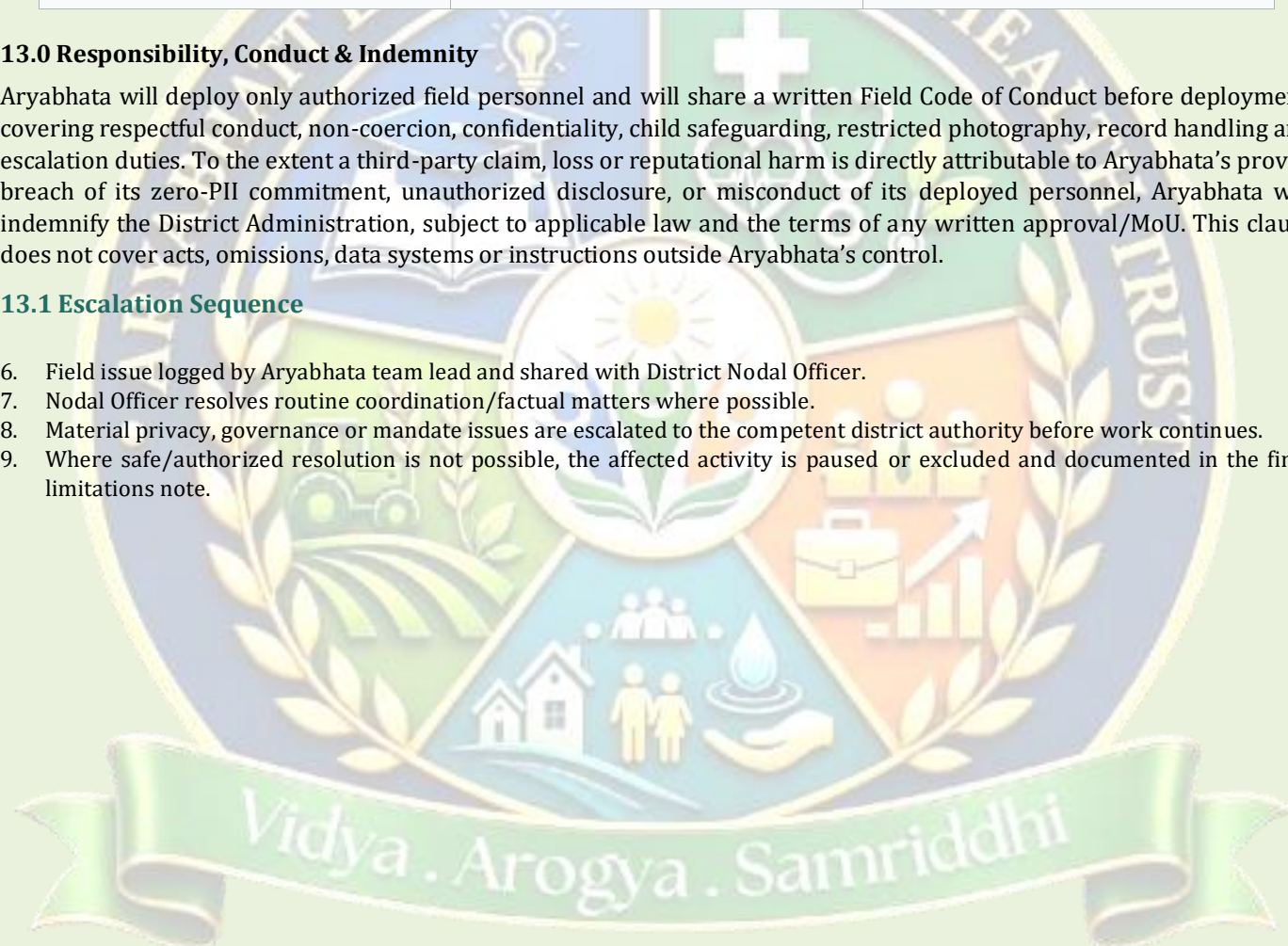
Risk	Control / Mitigation	Escalation Trigger
Data/privacy risk	Zero-PII design; anonymized tokens; controlled access; no public disclosure without approval	Any accidental receipt of PII or unauthorized access
Operational delay	Pre-agreed schedule; nodal officer coordination; capped sample; daily issue log	Access delay threatens 5–7 day completion
Factual disagreement	Exit briefing; “Factual Observations Shared” note; correction window	Material factual dispute remains unresolved
Perception of inspection/ranking	Written non-punitive protocol; no officer/institution ranking; system-level language	Frontline staff reasonably perceive coercive or disciplinary use
Reviewer conflict	Conflict declaration; district veto/alternative reviewer	Actual or perceived conflict identified
Funding expectation	Written no-cost/no-commitment clause and separate-approval boundary	Any party represents findings as sanctioned funding

13.0 Responsibility, Conduct & Indemnity

Aryabhata will deploy only authorized field personnel and will share a written Field Code of Conduct before deployment, covering respectful conduct, non-coercion, confidentiality, child safeguarding, restricted photography, record handling and escalation duties. To the extent a third-party claim, loss or reputational harm is directly attributable to Aryabhata’s proven breach of its zero-PII commitment, unauthorized disclosure, or misconduct of its deployed personnel, Aryabhata will indemnify the District Administration, subject to applicable law and the terms of any written approval/MoU. This clause does not cover acts, omissions, data systems or instructions outside Aryabhata’s control.

13.1 Escalation Sequence

- Field issue logged by Aryabhata team lead and shared with District Nodal Officer.
- Nodal Officer resolves routine coordination/factual matters where possible.
- Material privacy, governance or mandate issues are escalated to the competent district authority before work continues.
- Where safe/authorized resolution is not possible, the affected activity is paused or excluded and documented in the final limitations note.



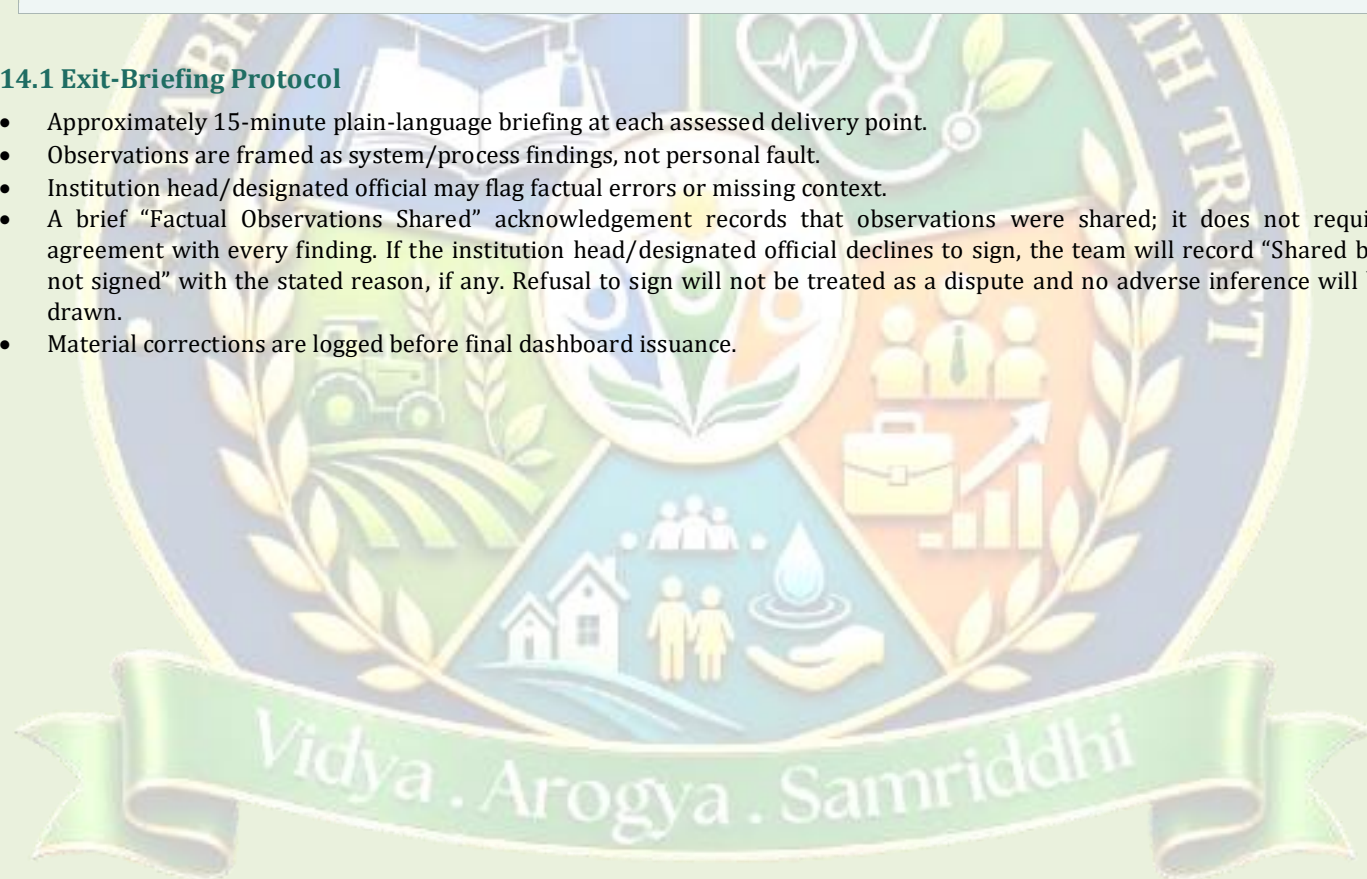
14. Field Timeline & Operational Protocol

Day	Activity	Key Output
Day 1	Kick-off, access confirmation, sample finalization, protocol briefing	Approved sample and field plan
Day 2	Document/process review and initial mapping	Evidence checklist and pathway map
Days 3–5	Field assessment across selected delivery points Field work will not be scheduled during school examinations, designated immunization/session days, or at Anganwadi Centres without a female Aryabhata team member present.	Verified observations and anonymized continuity evidence
Day 6	Delivery-point exit briefings; factual correction window; reviewer pack prepared	Factual observations shared / corrections logged
Day 7	Dashboard, Impact Passports and Priority Action Framework finalized; district handover	Decision-support package ready for District Administration / competent authority

Timeline Condition: The 5–7 working-day timeline assumes timely nomination of a Nodal Officer, access to selected delivery points, availability of authorized non-PII records, and no major public holiday/exam/emergency disruption. If these conditions are not met, the District Administration and Aryabhata may mutually revise the schedule without expanding scope.

14.1 Exit-Briefing Protocol

- Approximately 15-minute plain-language briefing at each assessed delivery point.
- Observations are framed as system/process findings, not personal fault.
- Institution head/designated official may flag factual errors or missing context.
- A brief “Factual Observations Shared” acknowledgement records that observations were shared; it does not require agreement with every finding. If the institution head/designated official declines to sign, the team will record “Shared but not signed” with the stated reason, if any. Refusal to sign will not be treated as a dispute and no adverse inference will be drawn.
- Material corrections are logged before final dashboard issuance.



15. Reporting, Handover & Use of Findings

At completion, Aryabhata will deliver the agreed Impact Passports, district convergence dashboard, Priority Action Framework and concise pilot report/briefing. The District Administration determines internal circulation and any further action. Aryabhata's role ends at the agreed handover unless a separate written assignment is issued.

Use	Position
Internal district review	Permitted for authorized administrative decision-making
Departmental follow-up	District may assign verified gaps to relevant departments/systems
Funding exploration	Possible only through separate applicable due process; pilot outputs are inputs, not sanctions
Public release / media use	Requires written authorization and additional privacy/reputational review
Replication / scale-up	Requires separate scope, approval, timeline and resource plan

15.1 Limitations to be Disclosed in Final Report

- Small, purposive pilot sample is not a district-wide statistical evaluation.
- ACS is a continuity indicator, not a causal impact measure.
- Findings reflect evidence available during the approved field window.
- No individual-level outcome claims will be made from anonymized token data alone.
- Recommendations remain subject to departmental verification, technical feasibility and competent-authority decision.



16. Approval Sought & Next Steps

Aryabhata respectfully seeks limited administrative approval to conduct the Phase 1 pilot under the safeguards and boundaries set out in this DPR.

Approval Requested	Specific Ask
1. Pilot permission	Permit the 5–7 working-day Phase 1 assessment at up to 10 jointly finalized delivery points.
2. Nodal coordination	Nominate one District Nodal Officer for scheduling, access and factual coordination.
3. Authorized non-PII access	Facilitate only the anonymized/pre-aggregated, non-PII records necessary for approved assessment questions.
4. Independent reviewer	Confirm Aryabhata's proposed reviewer, suggest an alternative, or exercise veto based on conflict/fit.
5. Handover meeting	Nominate the competent district audience for the Day-7 dashboard and action-framework briefing.

No Further Commitment: Approval of this pilot does not authorize any subsequent implementation, financial sanction, procurement, engagement, partnership or scale-up. Any such action requires a separate proposal and competent approval.

16.1 Proposed Administrative Sequence

10. District review of DPR and written permission/administrative direction.
11. Nomination of District Nodal Officer and confirmation of participating departments/systems.
12. Joint finalization of sample, access protocol, reviewer and field dates.
13. Five-to-seven working-day pilot execution.
14. Day-7 handover and decision-support briefing.



17. Annexures: Evidence Standards, Reviewer Declaration, Indicative MoU & Glossary

Annexure A — Indicative Impact Passport Fields

Field	Description
Delivery point code	Non-identifying code for the assessed point
Sector/system	Education / Health-RBSK / WCD-Anganwadi
Pathway assessed	Defined beneficiary service pathway or hand-off
Verified strengths	Evidence-backed practices supporting continuity
Verified gaps	Evidence-backed breaks/bottlenecks
ACS	0–100 continuity score with applicable-component count
Priority flags	Urgency, reach, feasibility
Responsible system	Department/system best placed to act
Immediate action	Low-cost/process correction where feasible
Escalation / resource need	Issue requiring higher-level or funded action

Annexure B — Minimum Evidence Standard

- Every reported gap should cite at least one observable/documentary/anonymized evidence source or be labelled “Not Verified”.
- No photographs of children or beneficiaries will be taken by the Aryabhata team. Photographs may be taken only of permitted process documents, wall displays, infrastructure or stock/register evidence after authorization, and only where no name, Aadhaar number, phone number, facial image, address or other PII is visible or captured.

Illustrative ACS scoring example: Documented Referral = 3/5 where a referral register/process exists but approximately 30% of sampled entries are incomplete. Evidence may include a photograph or extract of the relevant register/process page only after all PII is excluded and permission is obtained. The score records pathway evidence, not institutional merit.

Public-reporting safeguard: ACS bands will be shown only as aggregated/pathway heat-maps. No school, Anganwadi Centre, health touchpoint, officer or staff member will be publicly ranked or named against an ACS score.

- No Aadhaar number, phone number, name, home address, facial image, or other direct identifier is to be copied into Aryabhata working records.
- Reviewer validation should record material methodological limitations and disagreements, not merely endorse the final report.

Annexure C — Glossary

Term	Working Definition
Impact Passport	Single-page continuity summary of verified pathway observations and priority actions for one assessed delivery point; the term does not imply causal impact evaluation.
Zero-PII	Aryabhata receives no directly identifying beneficiary information and has no access to token-to-identity mapping; tokenized records are handled as restricted project data with no re-identification attempt.
Outcome-Readiness	Early evidence that the service pathway is capable of producing intended change; not a final impact claim.
Convergence Heat-map	Aggregate visual showing where inter-service continuity is strong or weak.
Decision-Support Dashboard	Consolidated district view for authorized administrative review.
District Nodal Officer	Official nominated for pilot coordination, access and factual resolution.
Independent Reviewer	External/independent expert who reviews methodology and anonymized analytical outputs.
ACS	Abhisaran Continuity Score; 0–100 pathway continuity indicator.
Priority Action Framework	Ranked register of verified gaps with urgency, reach, feasibility and ownership fields.

Term	Working Definition
DMF / PMKKKY / CSR	Potential future funding routes only where applicable and separately approved; not automatic outcomes of the pilot.

Annexure D — Indicative One-Page Pilot MoU / Administrative Understanding

This indicative template may be adapted by the District Administration. It does not replace any statutory approval or prescribed government format.

Parties	District Administration and Aryabhata Educational & Health Trust.
Purpose & Scope	Limited Phase 1 continuity pilot across up to 10 jointly finalized Education, Health/RBSK and WCD/Anganwadi delivery points; no ranking, inspection or disciplinary use.
Duration	Five to seven working days from mutually confirmed field commencement, subject to access conditions and any written schedule revision.
Zero-PII / Token Clause	Aryabhata will not access token-to-identity mapping, will not attempt re-identification, and will handle tokenized records as restricted project data. Any accidental PII receipt will trigger secure deletion/return and intimation to the Nodal Officer within two hours of discovery.
Photography / Safeguarding	No photographs of children or beneficiaries by Aryabhata. Only authorized non-PII process/infrastructure/document photographs may be taken. Field Code of Conduct and child-safeguarding rules apply.
Financial Liability	Nil financial ask to the District for this pilot. No reimbursement, procurement obligation, recurring liability, preferential funding right or automatic future engagement is created.
Data Retention & Handover	Unless otherwise directed in writing, tokenized/anonymized working files will be deleted within 30 days after Day-7 handover and a deletion certificate will be submitted.
Termination	Either side may terminate or suspend the pilot by written notice of 48 hours. Immediate suspension is permitted for privacy, safeguarding, legal, security or serious conduct concerns. On termination, Aryabhata will cease field activity and follow District directions on authorized data return/deletion.





About AEHT

From Public Programmes to Measurable Community Outcomes

AEHT is a Jharkhand-rooted implementation institution that converts government and CSR resources into field-level outcomes across agriculture, livelihoods, skilling, education, health, sanitation and women's empowerment. Since 2010, AEHT has worked through local centres, programme teams, partner institutions and its Strategic Management Unit to mobilise eligible communities, deliver practical training, support certification, placement and enterprise linkages, and maintain financial and administrative controls. Its role extends beyond conducting activities: AEHT connects programme intent with participation, completion, follow-through and verifiable evidence.

Delivery promise	Professional wording
Programme conversion	Translates approved schemes and CSR commitments into structured, completed interventions.
Livelihood outcomes	Links agriculture, beekeeping, mushroom, horticulture and trade skills with income pathways.
Inclusion at scale	Reaches rural, tribal, SC/ST, women and youth beneficiaries through field-based mobilisation.
Evidence & compliance	Supports beneficiary records, programme documentation and CSR-grade financial administration.

Our Vision:

ज्ञानेन प्रकाशः । आरोग्येन गरिमा । प्रकृत्या समन्वयः । सर्वजनहिताय समृद्धिः ॥

MEANING IN ENGLISH

"Enlightened by knowledge, dignified through health, living in harmony with nature, and advancing prosperity for the welfare of all."

Our Mission:

- English: We uphold integrity, transparency, innovation, and collaborative leadership as the foundation of responsible governance, ensuring that every initiative is guided by ethical values, institutional excellence, accountability, and a commitment to creating enduring and meaningful social change.
- हिन्दी: हम नैतिकता, पारदर्शिता, नवाचार एवं सहभागी नेतृत्व को उत्तरदायी सुशासन का आधार मानते हुए प्रत्येक पहल को नैतिक मूल्यों, संस्थागत उत्कृष्टता, जवाबदेही तथा लोककल्याण की भावना से संचालित करते हैं, ताकि समाज में दीर्घकालिक, सार्थक एवं सकारात्मक परिवर्तन का मार्ग प्रशस्त हो ।

Four Pillars, One Mission

A clear four-vertical architecture that presents AEHT as an integrated implementation partner.

Pillar	Profile-ready description
Resilient Agriculture & Rural Enterprise	Practical agriculture, allied livelihoods, beekeeping, horticulture and micro-enterprise pathways that strengthen household income.
Skills, Education & Employment	Vocational, digital and scheme-aligned training linked with certification, self-employment and placement support.
Community Health, Sanitation & Well-being	Preventive health outreach, hygiene awareness and community-based sanitation support in underserved locations.
Women's Agency & Inclusive Development	Skills, scholarships, enterprise and placement pathways for women, youth, tribal and other underserved communities.

Partner Institutions & Geography

Programme-to-Geography Matrix

Programme	Geography evidenced in profile	Beneficiary / delivery evidence	Information to add
Krishak Pathshala	Two blocks of Dhanbad	Farmers trained in agriculture, animal husbandry and fisheries	Block names, period, farmer count
Beekeeping	Hazaribagh, Giridih and Dhanbad	SC/ST farmers trained; boxes distributed	Household count, box count, income evidence
JSDM tailoring	Barkhata	200+ trained; focus on women's self-employment	District, batches, certification / placement
PMKVY 4.0	Not specified	250+ children; ~80% girls	Centre and district details
Beautician training	Hazaribagh evidenced through beneficiary case	Scholarship and salon placement support	Full programme coverage and cohort size
Horticulture / health support	Not specified	Training and village outreach described	Districts, dates and beneficiary counts

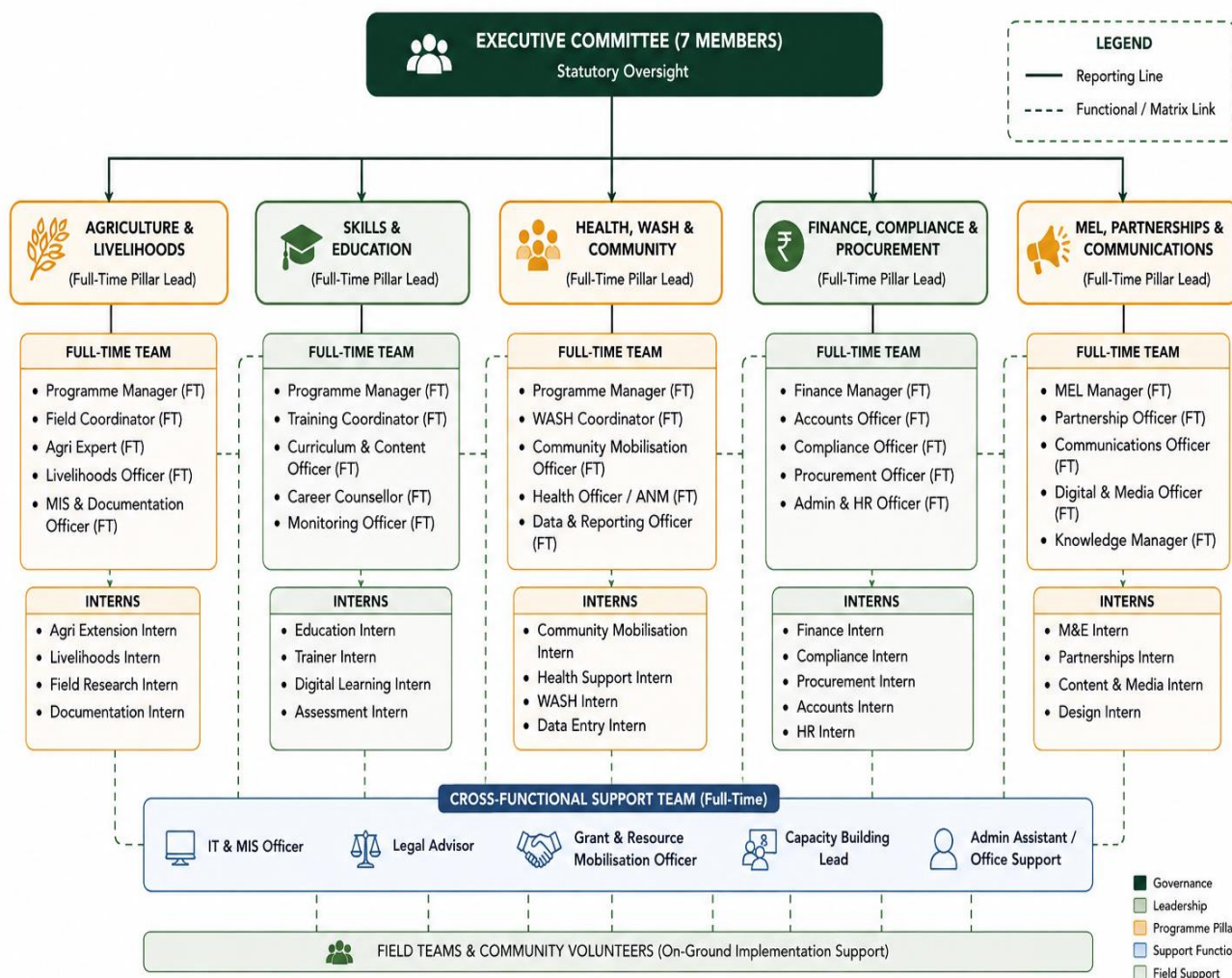
Governance

Existing statutory governance is shown first; all six management structures below are proposed alternatives.

Executive Committee	Name as stated in profile
President	Sri Vijay Kumar Paswan
Secretary	Sri Rohit Kumar
Treasurer	Smt. Kumari Munita Mehta
Member	Sri Tileshwar Prasad Mahto
Member	Sri Sibbu Kumar
Member	Sri Satendra Kumar Paswan
Member	Sri Srikant Kumar

PROGRAMME-VERTICAL MATRIX MODEL

For Technical Depth Across the Four Pillars



Function	Core full-time roles	Intern / associate pipeline
Agriculture & Livelihoods	Vertical lead; technical experts; coordinators	Agriculture / livelihood interns
Skills & Education	Vertical lead; centre and placement coordinators	Training / placement interns
Health, WASH & Community	Vertical lead; health/WASH coordinators	Public-health interns
Finance, Compliance & Procurement	Finance manager; accountant; procurement officer	Finance intern
MEL, Partnerships & Communications	MEL lead; CSR manager; data/comms executives	M&E and CSR interns

Registration & Compliance Tables

Registration Annexure & Document Tracker

Document	Identifier
Trust registration certificate/deed	8847/271/2010
PAN	AACTA9536F
12A order	AACTA9536F25PT01
80G order	AACTA9536F26PT01
CSR-1 acknowledgement	CSR00036783
NGO Darpan record	JH/2016/0103116
Udyam certificate	UDYAM-JH-11-0010369
GST certificate	20AACTA9536F2Z1

